

Bhagya Testing

30 Years/Female

Ref. by :

Partner : -

Report Ref. ID : BLR1218150

Patient ID : OHOBLR1218150

Collected : 14/04/2026 11:04 AM

Received : 14/04/2026 11:04 AM

Reported : 14/04/2026 11:09 AM



MC-6367



Test	Result	Bio. Ref. Range	Past Results
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BIOCHEMISTRY

Glucose

, Enzyme Method Glucose Oxidase-Peroxidase (GOD-POD)

3

Deficient: < 20
Insufficient: 20-30
Sufficient: 30-100
Potential Toxicity: > 100

Glycated Hemoglobin (HbA1C) | Whole Blood EDTA

Glycated Hemoglobin (HbA1C)

High-Performance Liquid Chromatography (HPLC)

5.0

%

Normal: < 5.7
Pre-Diabetes: 5.7-6.4
Diabetes: ⇒ 6.5

Mean Blood Glucose

Calculated

97

mg/dl

< 117

Lipid Profile | Serum

Cholesterol, Total

Cholesterol Esterase/Cholesterol Oxidase/Peroxidase

1

mg/dL

< 200

Triglycerides

Cholesterol Oxidase

2

mg/dl

< 150

High-Density Lipoprotein (HDL) Cholesterol

Cholesterol Esterase/Cholesterol Oxidase/Peroxidase

3

mg/dl

> 50

Non-High Density Lipoprotein (Non-HDL)

Cholesterol

Calculated

1

mg/dl

< 130

Low-Density Lipoprotein (LDL) Cholesterol

Calculated

2

mg/dl

< 100

Very Low-Density Lipoprotein (VLDL)

Cholesterol

Calculated

3

mg/dl

< 30

Cholesterol/High Density Lipoprotein (HDL)

Ratio

Calculated

1.0/mm³

3.3 - 4.4

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Low-Density Lipoprotein/High-Density Lipoprotein (LDL/HDL) Ratio Calculated	2.0	mg/dL 0.5 - 3	
High-Density Lipoprotein/Low-Density Lipoprotein (HDL/LDL) Ratio Calculated	3.0	Ratio > 0.4	

Remarks	Total Cholesterol (mg/dL)	Triglycerides (mg/dL)	LDL Cholesterol (mg/dL)
Optimal	<200	<150	<100
Above Optimal	-	-	100-129
Borderline	200-239	150-199	130-159
High	≥ 240	200-499	160-189
Very High	-	≥ 500	≥ 190

Lipid profile is a group test consisting of various lipids. Lipid profiles are generally collected with overnight fasting. However, recent guidelines have recommended non fasting samples for lipid profile for assessment of cardiovascular risk. The details for the study can be checked at <https://jamanetwork.com/journals/jamainternalmedicine/fullarticle/2733560>

In certain instances measurements in the same patient can show physiological and analytical variations. In such cases three serial samples at an interval of 1 week each are recommended for Total cholesterol, TG, HDL and LDL.

Cholesterol levels are increased in primary hypercholesterolemia; secondary hyperlipoproteinemia, including nephrotic syndrome; primary biliary cirrhosis; hypothyroidism; and in some cases, diabetes mellitus. Low cholesterol levels may be found in malnutrition, malabsorption, advanced malignancy, and hyperthyroidism.

Triglyceride levels are used in the diagnosis and treatment of patients with diabetes mellitus, nephrosis, liver obstruction, other diseases involving lipid metabolism, or various endocrine disorders.

High Density Lipoprotein (HDL) cholesterol levels is used to evaluate the risk of developing coronary heart disease (CHD). The risk of CHD increases with lower HDL cholesterol concentrations.

LDL (low-density lipoprotein) cholesterol level, sometimes called "bad" cholesterol, makes up most of our body's cholesterol. High levels of LDL cholesterol raise your risk for heart disease and stroke.

Very-low-density lipoprotein (VLDL) cholesterol is produced in the liver and released into the bloodstream to supply body tissues with triglycerides. High levels of VLDL cholesterol have been associated with the development of plaque deposits on artery walls, which narrow the passage and restrict blood flow.

Liver Function Test (LFT) | Serum

Bilirubin, Total Diazo Method	1	g/dL	0.2 - 1.3
Bilirubin, Direct Calculated	2	g/dL	0 - 0.3

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Bilirubin, Indirect Reflectance Spectrophotometry	<u>3.0</u> g/dL	0.1 - 1.1	
Aspartate Aminotransferase (AST) Multipoint-Rate/UV with Pyridoxal-5-Phosphate (P-5-P)	<u>1</u> U/L	14 - 36	
Alanine Transaminase (ALT) LDH, UV Kinetic	<u>2</u> U/L	<50	
Aspartate Aminotransferase/Alanine Transaminase (AST/ALT) Ratio Calculated	<u>3.0</u>	0.7 - 1.4	
Alkaline Phosphatase (ALP) Multipoint-Rate/UV with Pyridoxal-5-Phosphate (P-5-P)	<u>3</u> U/L	38 - 126	
Gamma-Glutamyl Transpeptidase (GGT) SZAZ Carboxylated Substrate	<u>1</u> U/L	12 - 43	
Protein Biuret	<u>2</u> mg/dl	6 - 8.3	
Albumin Bromo-Cresol Green	<u>3.0</u> g/dL	3.5 - 5	
Globulin Calculated	<u>1.0</u> g/dL	2.3 - 3.5	
Albumin/Globulin (A/G) Ratio Calculated	2.0	0.8 - 2	

In certain individuals, **total bilirubin** up to 2.0 mg/dl is considered normal. High bilirubin values can be due to jaundice.

Total bilirubin is invariably increased in jaundice. Causes of jaundice are prehepatic, resulting from various hemolytic diseases; hepatic, resulting from hepatocellular injury or obstruction; and posthepatic, resulting from obstruction of the hepatic or common bile ducts.

Increased **direct bilirubin** levels can occur in hepatobiliary disorders, including intrahepatic and extrahepatic biliary tree obstruction, liver cell damage, Dubin-Johnson syndrome, and Rotor syndrome.

High **indirect bilirubin** levels can occur in hemolytic disorders, Gilbert's syndrome, Crigler-Najjar syndrome, neonatal jaundice, and ineffective erythropoiesis.

High **Aspartate Aminotransferase** values can occur in Myocardial infarction, pulmonary emboli, skeletal muscle trauma, alcoholic cirrhosis, viral hepatitis, or drug-induced hepatitis.

Elevated **Alanine Aminotransferase** levels are seen in liver cell necrosis, hepatitis, hepatic cirrhosis, liver tumours, obstructive jaundice, Reye's syndrome, extensive trauma to skeletal muscle, myositis, myocarditis, or myocardial infarction.

High **alkaline phosphatase** levels can be due to primary and secondary hyperparathyroidism, Paget's disease of bone, carcinoma metastatic to the bone, osteogenic sarcoma, Hodgkin's disease, Hepatobiliary diseases involving cholestasis, inflammation, or cirrhosis.

ALP levels can also be elevated in fever and increased bone metabolism (e.g., in adolescents and during the healing of a fracture), in renal infarction and failure and in pregnancy complications.

Low **ALP** levels may occasionally be seen in hypothyroidism.

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Gamma-glutamyl transferase (GGT) is a sensitive indicator of hepatobiliary disease. It is useful in the diagnosis of obstructive jaundice and chronic alcoholic liver disease, in the follow-up of chronic alcoholics undergoing treatment, and in the detection of hepatotoxicity. GGT is more responsive to biliary obstruction than AST, ALT, or ALP.			
Total protein levels can be used to evaluate nutritional status.			
High protein concentrations can be due to dehydration, Waldenström's macroglobulinemia, multiple myeloma, hyperglobulinemia, granulomatous, and some tropical diseases.			
Low protein concentrations can be due to pregnancy, excessive intravenous fluid administration, cirrhosis or other liver diseases, chronic alcoholism, heart failure, nephrotic syndrome, glomerulonephritis, neoplasia, protein-losing enteropathies, malabsorption, and severe malnutrition.			
Increased albumin levels may indicate dehydration or hyperinfusion with albumin.			
Decreased albumin levels are found in rapid or over-hydration, severe malnutrition and malabsorption, severe diffuse liver necrosis, chronic active hepatitis, and neoplasia.			
Albumin is commonly reduced in chronic alcoholism, pregnancy, renal protein loss, thyroid dysfunction, peptic ulcer disease, and chronic inflammatory diseases.			
Globulin includes carrier proteins, enzymes, complement, and immunoglobulins. Most of these are synthesised in the liver, although immunoglobulins are synthesised by plasma cells.			
Increased globulin level usually results from an increase in immunoglobulins.			
Malnutrition and congenital immune deficiency can decrease globulin levels due to decreased synthesis. Nephrotic syndrome can cause decreased globulin levels due to protein loss through the kidney.			
AST/ALT Ratio > 2:1 (AST is two times higher than ALT) is indicative of alcoholic liver disease.			
AST/ALT Ratio < 1:1 (ALT is higher than AST) indicates non-alcoholic fatty liver disease.			

Kidney Function Test (KFT) | Serum

Urea Urease	13.0	mg/dl	15 - 36
Creatinine Twopoint-Rate-Creatinine Aminohydrolase	23.00	mg/dl	0.52 - 1.04
Blood Urea Nitrogen (BUN) Calculated	6.1	mg/dl	6 - 20
Blood Urea Nitrogen (BUN)/Creatinine Ratio Calculated	0.3	Ratio	10 - 20
Uric Acid Uricase	3	mg/dl	2.5 - 6.2
Calcium Arsenazo Method	1	mg/dL	8.4 - 10.2
Phosphorus Phosphomolybdate Formation	2	mg/dLI	2.5 - 4.5

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Estimated Glomerular Filtration Rate (eGFR) Twopoint-Rate-Creatinine Aminohydrolase/Calculation	2	Normal: $\Rightarrow$ 90 Mild decrease: 60-89 Mild moderate decrease: 45-59 Severe decrease: 15-29 End stage kidney disease: < 15 ml/min/1.73 sq m	
Electrolytes			
Sodium Direct ISE	3.00	mmol/LL	137 - 145
Potassium Direct ISE	4.0	mmol/L	3.5 - 5.5
Chloride Direct ISE	333	mmol	98 - 107

High **sodium** levels can be caused by dehydration, disorder of the adrenal glands, diarrhoea, diuretics, kidney disease, and diabetes insipidus.
Low **sodium** levels are caused by diarrhoea, vomiting, kidney disease, liver failure, Addison's disease, and malnutrition.

High **potassium** levels may be due to low aldosterone, kidney failure, metabolic or respiratory acidosis.
Low **potassium** levels may be due to diarrhoea, vomiting, diuretics, and high aldosterone.

Chloride is used to help diagnose conditions related to an imbalance of acids or fluids in the body. Certain medicines, such as antacids, can also cause abnormal results.

High **chloride** levels may be due to Addison's disease, metabolic acidosis, respiratory alkalosis, and renal tubular acidosis.

Low **chloride** levels may be due to burns, CHF, metabolic alkalosis, vomiting, dehydration, and respiratory acidosis(compensated).

High **urea** and **BUN** levels are suggestive of poor kidney function due to acute or chronic kidney diseases, decreased blood flow to the kidneys as in congestive heart failure, shock, stress, recent heart attack or severe burns, bleeding from the gastrointestinal tract, conditions that obstruct urine flow or dehydration.

Low **urea** and **BUN** levels are uncommon and are not usually a cause for concern. They can be seen in severe liver disease or malnutrition but are not used to diagnose or monitor these conditions. Low urea levels are also seen in normal pregnancy.

Creatinine is elevated in kidney disease, damage, infection, urinary tract obstruction, reduced blood flow to the kidneys in case of shock, congestive heart failure, complications of diabetes.

High levels of **uric acid** are seen in kidney disease, pre-eclampsia, purine-rich food, alcoholism, and side effects of cancer treatment.

Low **calcium** levels may be due to hypoparathyroidism, kidney failure, pancreatitis, malnutrition, or a disorder in calcium absorption.

High **calcium** levels may be due to hyperparathyroidism, hyperthyroidism, sarcoidosis, drugs like diuretics, and excessive calcium supplementation.

High **phosphorus** levels can be due to dehydration, hypoparathyroidism, hypervitaminosis D, metastases to bone, sarcoidosis, pulmonary embolism, renal failure, or diabetes mellitus with ketosis.

Low **phosphorus** levels can be caused by hyperparathyroidism, high calcium levels, sepsis, vitamin D deficiency, renal tubular disorders, chronic hemodialysis, vomiting, or occasionally decreased dietary phosphate intake.

Chronic Kidney Disease often has no symptoms until the later stages. So, reliable estimates of GFR are important for identifying the disease as early as possible.

Factors that can affect **eGFR** include pregnancy, being over the age of 70, unusual muscle mass, cirrhosis, nephrotic syndrome, a past solid organ transplant, and some medications.

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Approved By

Dr. Sayantani Sarkar
MBBS, MD (Pathology)
Pathologist

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IMMUNOLOGY

Vitamin B12, Cyanocobalamin Chemiluminescent Microparticle Immunoassay (CMIA)	444	pg/mL	187 - 883
Vitamin D, 25-Hydroxy Chemiluminescent Microparticle Immunoassay (CMIA)	34	ng/mL	Deficient: < 20 Insufficient: 20-30 Sufficient: 30-100 Potential Toxicity: > 100

Thyroid Function Test (TFT) | Serum

Triiodothyronine (T3), Total Chemiluminescent Microparticle Immunoassay (CMIA)	1	ng/mL	0.35 - 1.93
Thyroxine (T4), Total Chemiluminescent Microparticle Immunoassay (CMIA)	2	µg/dL	4.87 - 11.72
Thyroid Stimulating Hormone (TSH) Chemiluminescent Microparticle Immunoassay (CMIA)	3	µIU/mL	0.35 - 4.94

Approved By

Dr. Sayantani Sarkar
MBBS, MD (Pathology)
Pathologist

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MICROBIOLOGY

Culture & Sensitivity - Aerobic, Urine

Report Status : Final Report

Incubation period : 48 hrs of incubation

Specimen : Aspirated fluid

Culture

Pathogen

Colony Count

Candida boidinii

Sensitivity for 'Candida boidinii'

Antimicrobial	Sensitivity	MIC Value
Amphotercine - B	Sensitive	34
Caspofungin	Intermediate	44
Fluconazole	Resistant	23
Ketoconazole	Susceptible	22

Interpretation & Notes:

- Interpretation done as per CLSI M100.
- A negative culture (or) No growth report indicates the absence of pathogenic organisms.
- False-negative results can be caused by low number of microorganisms, prior antimicrobial treatment, the fastidious nature of the infective organism.
- A positive culture report may mean that a patient is having a bacterial or fungal infection.
- False-positive culture report can result from contamination of the specimen with skin microbiome.

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Approved By

Dr. Trupthi
MBBS, MD (Microbiology)
Microbiologist

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Reviewed By

Dr. Jushmita Pathak
MBBS, MD (Pathology)
Pathologist

Dr. Joan Maria Vynetta D
MBBS, MD (Pathology)
Pathologist

CONDITIONS OF LABORATORY TESTING & REPORTING

- It is presumed that the test sample belongs to the patient named or identified in the test requisition form. Test results released pertain to the specimen submitted.
- Laboratory investigations are only a tool to facilitate arriving at a diagnosis and should be clinically correlated by the Referring Physician.
- All tests are performed and reported as per the turnaround time stated in the Orange Health Labs Directory of Services (DOS).
- Orange Health Labs confirms that all tests have been performed or assayed with the highest quality standards, clinical safety & technical integrity.
- All test results are dependent on the quality of the sample received by the Laboratory and the assay technology.
- Report delivery may be delayed due to unforeseen circumstances. Inconvenience is regretted.
- A requested test might not be performed if:
 - The specimen received is insufficient or inappropriate, or the specimen quality is unsatisfactory
 - Incorrect specimen type
 - Request for testing is withdrawn by the ordering doctor or patient
 - There is a discrepancy between the label on the specimen container and the name on the test requisition form
- Test results may show interlaboratory variations.
- Test results are not valid for medico-legal purposes.
- This is a computer-generated medical diagnostic report that has been validated by an Authorized Medical Practitioner/Doctor. The report does not need a physical signature.

REPORT PRINTOUT/HARDCOPY

- Patients can collect free report printouts from our Lab or the nearest Collection Centre.
- Hard copies can be delivered within 24 hours for ₹200/report (non-refundable after dispatch).
- For assistance, contact Orange Support at +91-9008111144.